CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

BRAF / MEK + PD-(L)1: 2 molecules stopped early, all small molecules

3 stopped trials, but 2 molecules — all small molecules: Cobimetinib and Binimetinib. Against 5.3% for PD-(L)1 combinations without BRAF / MEK and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

4.7%
3 of 64 closed trials (95% CI 1.6%–12.9%)

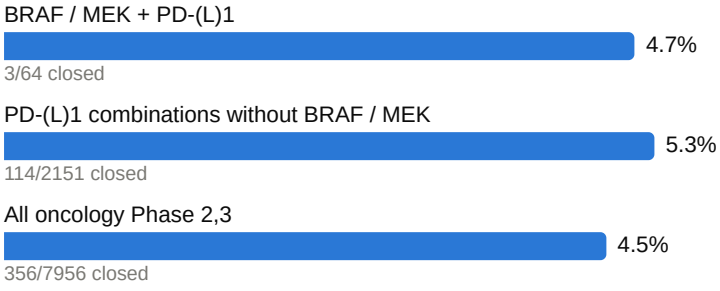
1 / 0 / 1
efficacy / safety / both (adds to 3)

39.8%
of 161 trials have closed · 97 still open or unresolved

1.6%
stopped for a biological reason within 3 years of starting (95% CI 0.4%–6.4%; 98 still at risk; 1.5% area-wide)

3 of 3 **Completed and missed the primary endpoint.** Of 45 completed trials in this segment, 3 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 3 missed and 0 met. Counted separately and never in the rate above; a floor, not a rate. 17 of the 40 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 190 distinct experimental drugs across 161 trials (small molecule 161, antibody 151, unknown 27, adc 12). Most tested: REGORAFENIB (46), COBIMETINIB (42), ATEZOLIZUMAB (42), NIVOLUMAB (32), TRAMETINIB (23). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 3 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 3.3%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 4 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 10.9%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	22	4.5%	0.8%–21.8%
2018–2020	2	25	8.0%	2.2%–25.0%
2021–2024	0	17	0.0%	0.0%–18.4%

Where the stops sit

Efficacy and safety: 1, Efficacy / futility: 1, Benefit–risk: 1.
Sponsors with most stops: Columbia University (1), Pfizer (1), Roche (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04214418	2	Columbia University	COBIMETINIB, HYDROXYCHLOROQUINE, ATEZOLIZUMAB	2022-04	Efficacy + safety	Toxicity and Lack of Efficacy

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT03637491	2	Pfizer PFE	AVELUMAB, BINIMETINIB, TALAZOPARIB	2020-12	Efficacy	Available clinical data has shown limited anti-tumor activity and reaching target study drug dose levels may not be feasible.
NCT03273153	3	Roche	COBIMETINIB, ATEZOLIZUMAB	2019-04	Benefit–risk	This study was terminated due to benefit/risk analysis.

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 39.8% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.